Case Study — Visioning an AI-Native Clinical Platform

From vision workshops to a shipped end-to-end visit experience

2026-09-20 · @Someone

The short version

A clinical assistant used by primary care physicians had grown into four disconnected tabs and a pile of AI point solutions. It was being used as an end-of-visit administrative checklist, which was neither what it was for nor where its value was.

I led the visioning for its redesign. I facilitated workshops with the chief product officer, the design director, and clinical stakeholders, synthesized that into an end-to-end vision prototype, and used the prototype as the reference for a platform-wide transformation.

That vision then became an operating plan. Working with product leadership, I broke it into workstreams sized by unknown rather than by feature, sequenced them across two quarters, and helped form development teams around the workflows. I held the experience architecture while a new design system was built underneath it and each team pushed its piece forward against real engineering constraints.

The MVP shipped in June. When the parallel teams had produced fourteen competing prototypes, I converged them back into a single end-to-end flow for alpha.

Vision → roadmap → team structure → architecture → convergence → ship.

Starting conditions

The product served primary care physicians seeing 40+ patients a day, in a value-based care model where the quality of the clinical encounter drives both outcomes and reimbursement.

It had accumulated real capability — a scribe, a patient-data chat, a visit briefing, a task system — and each worked. The problem was that none of them knew about the others. A physician moved between Briefing, Tasks, Notes, and Chat as separate destinations, and

the product's center of gravity had drifted to *after* the visit: a checklist you cleared once the patient had left.

That is the opposite of where the value is. The moment worth designing for is the visit itself.

Why this needed a vision rather than a backlog. Every individual improvement anyone proposed made one tab better. None of them addressed the fact that the tabs were the problem. Fixing this iteratively would have meant optimizing a structure we had already concluded was wrong — and the AI capabilities arriving over the next year would not fit into it at all.

The transformation had to be defined whole, then decomposed. That is a different exercise from planning a quarter, and it needed the CPO, design leadership, and clinical stakeholders in a room agreeing on the destination before anyone argued about sequence.

Visioning

I facilitated a series of vision workshops with the chief product officer, the design director, and clinical stakeholders.

The composition was the point. Product leadership held the strategic bet and the constraints. Clinical stakeholders held the thing no one else could supply — what actually happens in the room, what a physician will and will not tolerate mid-visit, and which of our assumptions were quietly wrong. Design leadership held the question of what the organization could realistically absorb.

Getting those three views into the same artifact, early, was what made the later roadmap defensible. It is also what kept the vision from being a design fiction: every element had someone in the room who could say whether it was clinically real.

The method was hypothesis-first. Rather than presenting a destination for approval, I put each stage of the proposed experience in front of the group with its assumption stated plainly and its unknowns listed — what we thought physicians would do, and what we could not yet answer. That framing invites disagreement instead of endorsement, which is the only way clinical input changes a design rather than ratifying it.

> **Worth confirming.** I've inferred the workshop cadence from the artifacts rather than your account — tell me how many sessions and over what period, and I'll make it specific. If you remember a particular moment where clinical pushback redirected the vision, that would be the strongest single paragraph in this case study.

What came out of it was a shared transformation statement, later written into the project plan in From→To form:

From	To
Fragmented tabs — Briefing, Tasks, Notes, Chat	A single unified workflow
Tasks as an administrative checklist	The clinical agenda as the plan for the visit
Manual and step-based	Continuous and ambient-driven
AI point solutions	Deeply embedded AI infrastructure
Desktop, workflow-heavy	Mobile-first — portable, ambient, embeddable in the EHR

Five lines, each one a bet with an owner. This table did more work over the following year than any deck I have made.

The vision prototype

I took the workshop output and built an end-to-end vision prototype: the whole visit, pre-visit through post-visit, as one continuous experience rather than a set of screens.

A prototype rather than a deck, for a specific reason. A slide showing "unified workflow" gets nodded at and then interpreted five different ways by five teams. A prototype you can move through forces the hard questions into the open — what happens between steps, what the physician sees when the ambient layer is listening, what the agenda looks like when a patient has nothing going on. It converts agreement-in-principle into agreement-in-detail, which is the only kind that survives handoff.

It also became the artifact everything downstream pointed at. When eight workstreams are running in parallel, the expensive failure is each one optimizing locally until the pieces no longer compose. A shared reference for the destination is what prevents that.

How it was built. The flow ran in four stages — Schedule → Visit → Visit + Task → Visit completion — drawn at three breakpoints (XS, MD, XL). Drawing the small breakpoint first, and completely, is what made "mobile-first" a constraint the teams had to design against rather than an aspiration in a deck.

Each stage carried three annotations, and this is the part I would show a hiring manager:

- **A user story** — what the physician is doing and why they are here.
- **A hypothesis** — what we believed would happen, stated so it could be wrong. For the agenda stage: that a glanceable list of prioritized assessment items would let physicians click deeper only where something was new or surprising.
- **Open questions** — what we did not know yet, written into the artifact instead of resolved by assertion.

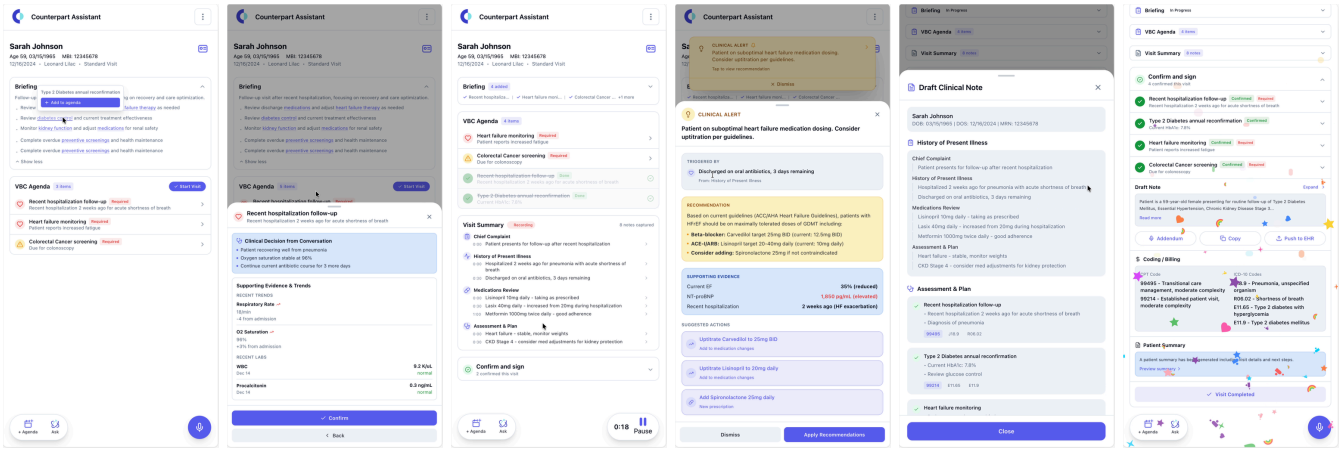
And an explicit Included / Not Included boundary per stage. This is where the vision became a roadmap. Rather than presenting a vision and negotiating scope afterward, each stage stated what the MVP would carry and what it would not — brief and scribe in, add-a-task out. The scoping conversation happened inside the artifact, at the moment the design made the tradeoff legible.

The review loop is visible in the file. Stakeholder feedback was captured inline against the frames rather than collected in a separate doc: questions about whether the visit and patient information were correctly grouped in the header, a note that the design showed the member ID but not date of birth when date of birth is what physicians actually use to identify a patient, a challenge to putting primary content in a bottom sheet, and a concern that a panel resizing at every question would be disorienting mid-visit.

That last set is worth pointing at directly. Those are not polish notes — they are practitioners telling us the design did not match how the room works, recorded against the exact frame they refer to.

The one-year vision

Beyond the core flow, I produced walkthroughs of nine capabilities as they would work a year out — recorded as silent screen captures, almost all of them at phone dimensions. Together they are the clearest statement of what the platform was for.

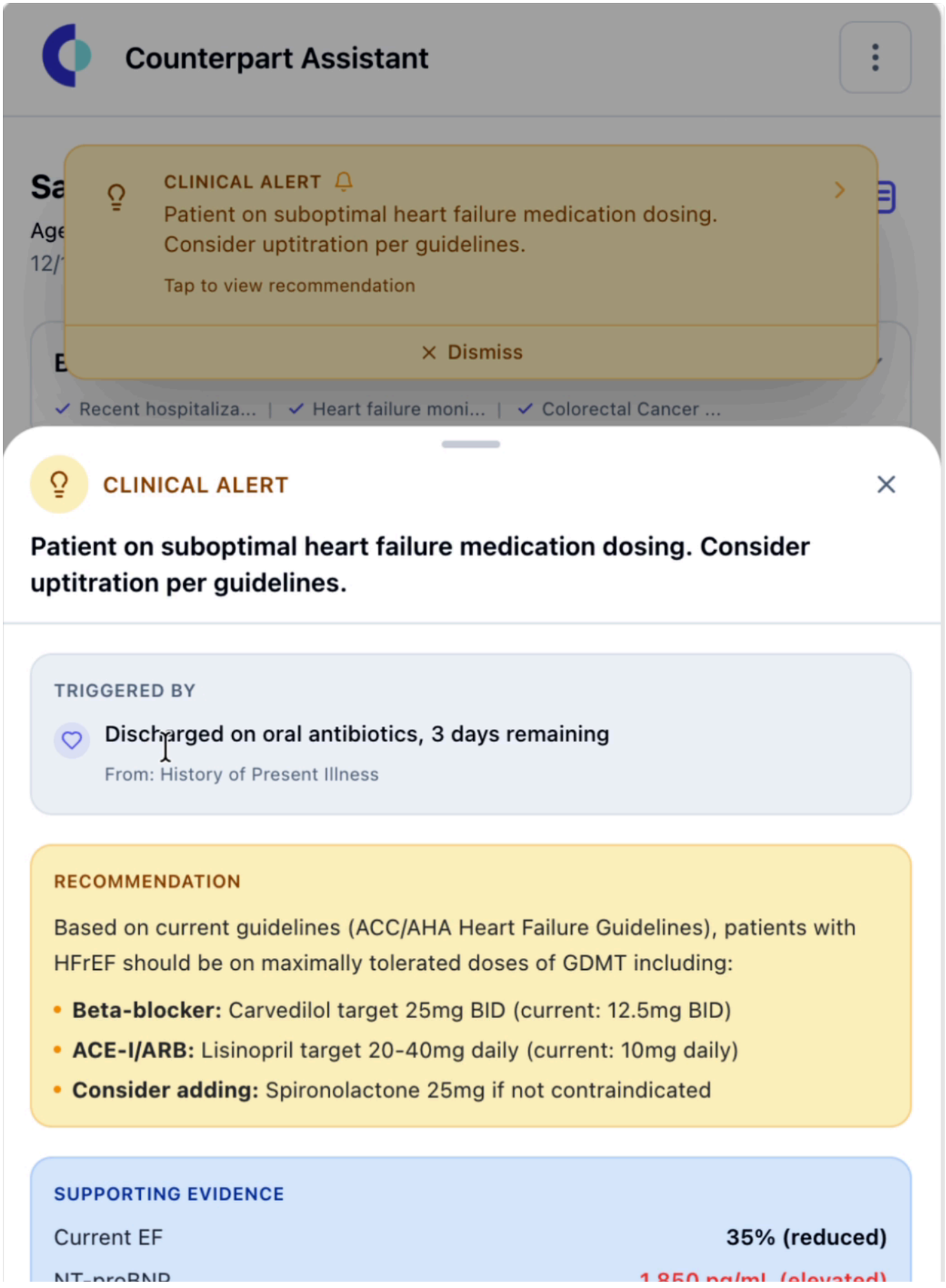


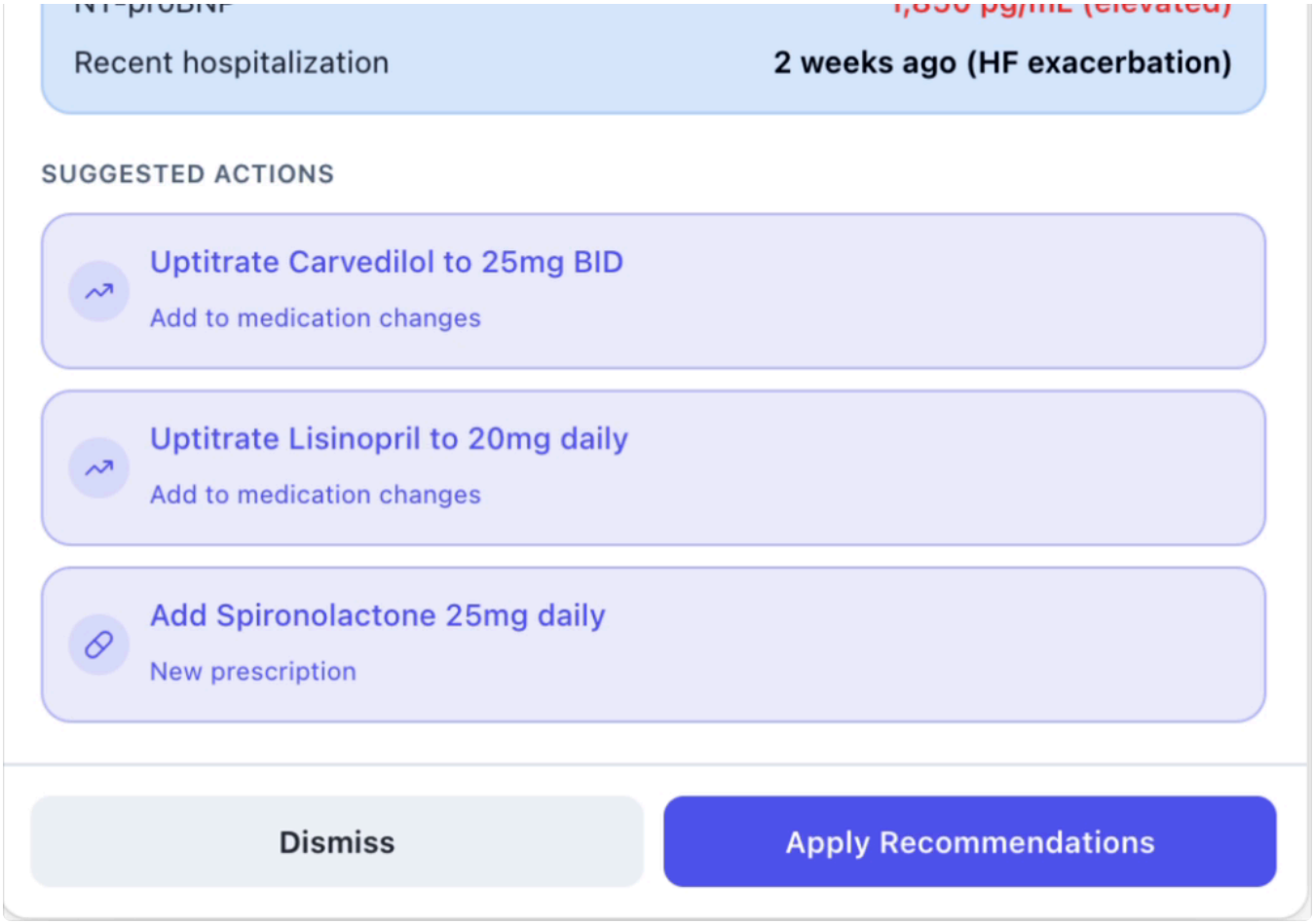
Capability	What it demonstrates
Briefing	Short, linked context — tap a clinical term to add it to the agenda
VBC agenda	The agenda as the visit's spine, with evidence drawers per item
Start the visit (with and without scribe)	The same workflow whether or not ambient capture is running
Ambient scribe integration	The visit note assembling itself as the conversation happens
Contextual alerts	Guideline-grounded alerts firing from what was just said
Patient data	Longitudinal record as a reference layer, not a destination
Sign and complete	Note, coding, and patient summary generated from the visit
Browser	The assistant running as a panel beside an EHR

Three things in these that matter more than the individual screens.

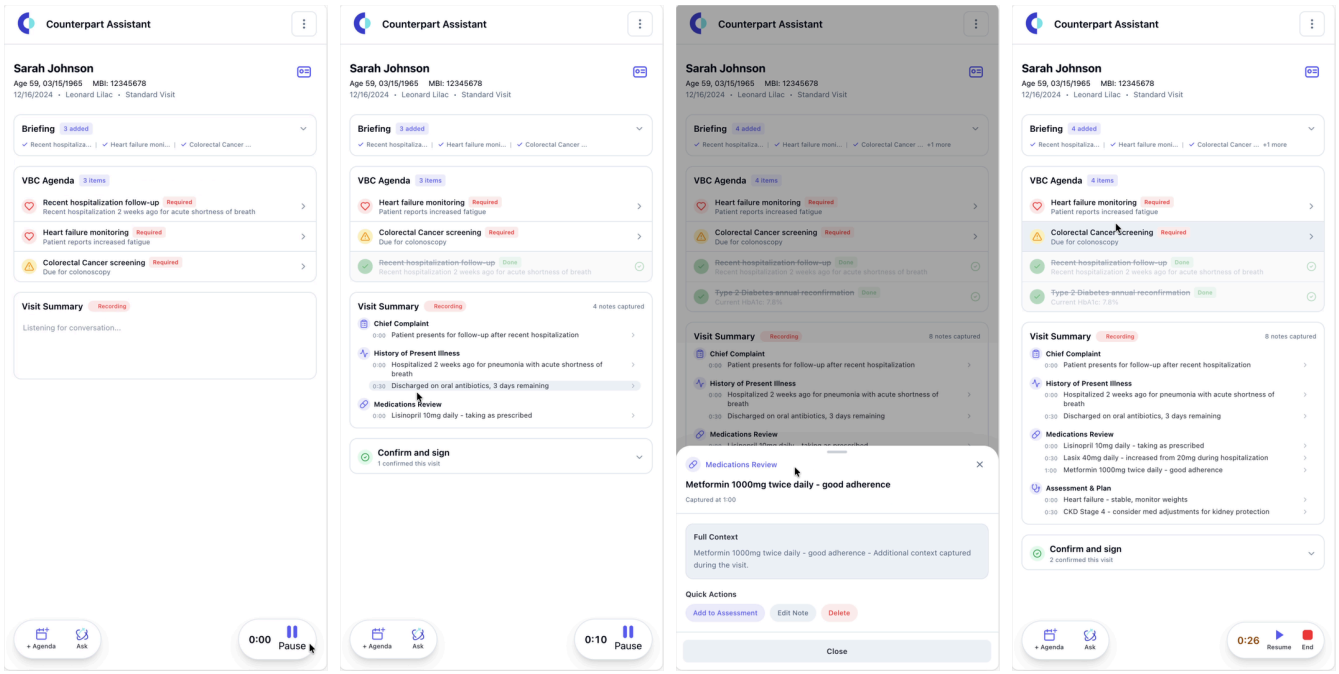
The whole thing was drawn at phone size. Not adapted to it — authored there. That is what made mobile-first a constraint the teams inherited rather than a principle they could quietly drop when desktop got hard.

Evidence and traceability were designed in from the start. Alerts state what triggered them and cite the guideline they come from. Agenda items open to supporting labs and trends. The pattern the team later adopted as a principle — surface data, show your work, never editorialize — was in the vision before it was a decision anyone had to make.

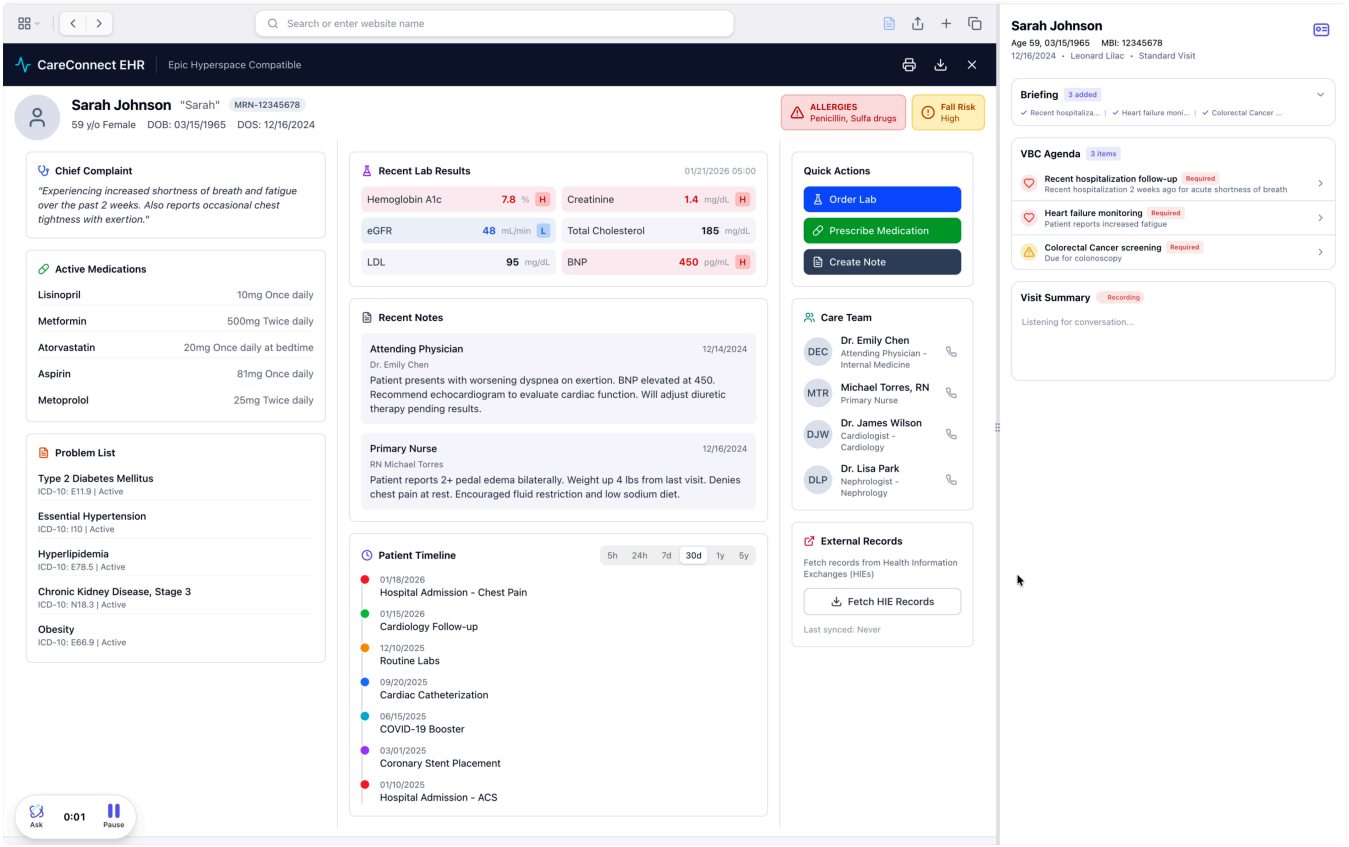

Counterpart Assistant
CLINICAL ALERT
Patient on suboptimal heart failure medication dosing.
Consider uptitration per guidelines.
Tap to view recommendation
Dismiss
Recent hospitaliza...
Heart failure moni...
Colorectal Cancer ...
CLINICAL ALERT
Patient on suboptimal heart failure medication dosing. Consider uptitration per guidelines.
TRIGGERED BY
Discharged on oral antibiotics, 3 days remaining
From: History of Present Illness
RECOMMENDATION
Based on current guidelines (ACC/AHA Heart Failure Guidelines), patients with HFrEF should be on maximally tolerated doses of GDMT including:
Beta-blocker: Carvedilol target 25mg BID (current: 12.5mg BID)
ACE-I/ARB: Lisinopril target 20-40mg daily (current: 10mg daily)
Consider adding: Spironolactone 25mg if not contraindicated
SUPPORTING EVIDENCE
Current EF
35% (reduced)



The ambient layer drives the agenda rather than sitting beside it. Items complete themselves as topics are discussed, the note builds in real time, and the physician confirms rather than transcribes. This is the whole "manual and step-based → continuous and ambient-driven" line of the transformation, made concrete enough to argue with.



The browser walkthrough makes the platform ambition explicit: the assistant running as a panel alongside an EHR, with the patient record, care team, labs, and timeline reachable without leaving the visit. That is the EHR-embeddable claim, demonstrated rather than asserted.



Where it went. Alongside the core flow I ran parallel desktop explorations — visit note with a patient chart sidebar, visit note with assessment detail, and two navigation-sidebar variants — so the wide-screen question was posed as a real choice rather than a stretched mobile layout.

The vision's structure is visible in what shipped: the agenda as the spine of the visit, the briefing folded into it as context rather than a separate tab, ambient capture running alongside rather than as a separate mode, and completion as a light sign-off rather than a second workflow.

Several decisions the team formally aligned on months later were already resolved here — brief items treated as agenda items even without an underlying task, tiered clinical alerting, review as sign-off rather than a working surface, and evidence shown inline with every finding. A vision artifact earns its keep when the arguments it settles stay settled.

From vision to roadmap

A vision that does not decompose is just a mood. The next job was turning it into work that teams could own.

I worked with product leadership to break the transformation into workstreams and sequence them. The organizing choice that mattered: **we sized workstreams by unknown, not by feature.** Each one was assessed on how much we did not know and how much impact it carried, and that drove sequencing and staffing rather than scope or visible surface area.

Workstream	Core transformation	Unknowns / Impact
Clinical agenda	Connective tissue between briefing and tasks; concise, glanceable	**High / High**
Task system rearchitecture	Flexible, conversational, adaptive; new task types	Extremely high / High
Ambient capabilities	Scribe-to-task autofill; real-time completion and orchestration	Medium / High
Patient record + chat	Source-of-truth record; AI-native chart	Medium / High
Agent infrastructure	Sub-agents, tooling, latency, guideline access	High / High

Workstream	Core transformation	Unknowns / Impact
Design system overhaul	Mobile-first UI; reimagined desktop IA	Medium / Medium
Provider schedule	Real schedule with times; agenda entry point	Low / Medium
Checkout flow	Unified, simplified post-visit completion	Low / Low

That framing produced decisions that would have been contentious otherwise. The task system carried the highest uncertainty of anything on the list, so it was explicitly deferred out of the MVP rather than attempted and botched. The clinical agenda, high on both axes, got prototyping capacity early and a deliberately small validation target: one deeply engaged physician using it for real visits by end of quarter. The checkout flow, low on both, was scheduled but never allowed to consume attention.

Teams formed around workflows, not components. Each workstream got a pod that owned its slice end to end. This is the part I would defend hardest — component-owned teams produce locally optimal pieces that nobody is accountable for composing. Workflow-owned teams argue about the seams, which is where the experience actually lives.

Governance had two layers. A weekly working group with a representative from each pod plus design leadership, for report-outs, cross-pod blockers, and decisions. A monthly cross-pillar alignment with the CPO and clinical lead for codesign and direction. The weekly kept the pods composable; the monthly kept the whole thing pointed at the vision.

Holding the architecture

Once the pods were running, my role changed. I was no longer producing the design — I was keeping it coherent while other people produced it.

The experience architecture was the through-line: how the agenda relates to the briefing, what the ambient layer is allowed to do to the screen, where a physician's attention is supposed to be at each moment, which surface owns which decision. Individual teams could design well inside their slice and still break the whole if those relationships drifted.

A new design system was being built underneath us simultaneously, aimed at the mobile-first, AI-native direction rather than retrofitted from the existing desktop UI. That created a genuine tension worth naming: the pods needed to move fast on patterns that were still being defined, and the design system team needed real usage to define them against. We ran them in parallel deliberately, accepting that some prototype work would be rebuilt once the system firmed up.

The build approach made that affordable. Work ran in an isolated module sandbox — separate data layer and component library, feature-flagged, prototype-first, with AI-assisted design-to-code so engineering could move at design's pace. The explicit agreement was that this code was disposable. Whatever proved out would be rebuilt to production standards; whatever did not would be deleted without ceremony.

That one agreement is what let eight workstreams explore in parallel without the usual paralysis about committing to the wrong thing.

Convergence

Parallel exploration has a predictable cost, and we paid it on schedule. By midsummer the pods had produced roughly fourteen prototypes of the live visit experience — each defensible on its own, none of them the same product.

That is not a failure of the approach. It is what the approach buys: fourteen explorations of a genuinely uncertain problem, run cheaply, in the open. But somebody has to converge them, and convergence is a different skill from exploration.

My work for the back half of the summer was coalescing them into a single end-to-end flow that could go to alpha:

- **Unifying the design system across surfaces** — the prototypes had drifted into different backgrounds, spacing, and component behavior. Shared foundations first, because visual inconsistency reads as conceptual inconsistency to a reviewer and makes real differences impossible to evaluate.
- **Making the layout genuinely responsive** rather than maintaining separate narrow and wide explorations that were slowly diverging.
- **Reconciling interaction patterns** where two teams had solved the same problem differently — click behavior, disclosure, how the ambient layer surfaces what it hears.
- **Preserving what was actually good** in each exploration rather than picking a winner and discarding the rest.

That last one produced the hardest lesson of the phase, and it is worth stating as a principle: **when convergence work names a specific prototype as the quality bar, branch that exact artifact and apply new context to it additively.** Rebuilding its decisions inside a newer, cleaner shell will faithfully preserve the requirements and quietly erase the experience everyone was pointing at. We learned that the expensive way once. It is now written down as a practice.

The converged flow is what went to the alpha cohort.

Outcome

Shipped. The MVP went engineering-complete in June — redesigned visit page built on the clinical agenda, assessments restructured, scribe integrated into the visit rather than bolted beside it, and a two-step completion flow with no regression on payment or submission. The converged flow followed to a curated alpha cohort of six physicians across two health plans.

Measured. Deep engagement rose from 34% to 44% over the quarter, against a 55% target. Clinical-value satisfaction held at 85%. A best-in-class UX index moved from −0.4 to +0.5.

Honestly assessed. The central bet — that a clinical agenda beats an administrative checklist — was still unproven when I left. It was the highest-risk item on the roadmap from the beginning and it was correctly identified as such; the alpha existed precisely to test it. The organizational transformation was real and delivered. The clinical outcome was still in flight.

> **Needs your input.** These figures come from the pillar strategy doc and are quarter-level, not attributable solely to this work. If you have anything closer to the visit experience itself — alpha adoption, time-in-visit, physician quotes — it would be stronger than borrowed pillar metrics.

Reflections

Size work by unknown, not by feature. The single most useful decision in the roadmap. It made deferring the highest-uncertainty workstream an obvious call rather than a fight, and it put prototyping capacity where the risk actually was.

A prototype is a better vision artifact than a deck. It forces the between-the-screens questions into the open early, and it gives eight parallel teams something unambiguous to point at.

Parallel exploration needs a convergence plan from day one. We planned the divergence carefully and improvised the convergence. Fourteen prototypes was the predictable result of the structure we built, and I should have designed the merge path at the same time I designed the split — including who owns it and what the criteria are.

Disposable code is a design permission, not an engineering shortcut. Agreeing up front that the sandbox was throwaway removed the fear that made every earlier exploration cautious. It is the cheapest thing on this list and it changed how the teams worked.

Hold the architecture loosely enough that teams can surprise you. Several of the best ideas in the final flow came from pods solving their own problems in ways the vision prototype had not anticipated. My job was to keep the relationships coherent, not to make the outcome match my original drawing.

Production notes

Working notes — not part of the case study.

Evidence status. The roadmap, workstream, governance, and outcome sections are grounded in project documents. The visioning and vision-prototype sections are written from your account alone and are the two thinnest parts of the draft — which is unfortunate, because they are the parts that make this a vision case study rather than a delivery one.

Section	Evidence	Gap
Starting conditions	Project plan, pillar strategy	None
Visioning	From→To table; hypothesis-first method visible in prototype annotations	Session count and dates; a moment clinical input redirected the work
Vision prototype	**Figma core flow + nine capability walkthroughs**	None — this is now the best-evidenced section
Roadmap	Project plan, documented in full	Your specific authorship role

Section	Evidence	Gap
Architecture	Project plan, design brief	None
Convergence	Repo file record, retro entry	None
Outcome	Pillar strategy metrics	Metrics closer to this work

Figma is connected and the Core Definitions file reads fine. I can pull frames, structure, and screenshots on request. Two limits: the sandbox network blocks figma.com, so images come inline rather than downloaded, and the older Counterpart-org file keys from the repo return no-access — this file works because it's in an account you still have.

Whose annotations? The open questions on the frames fall into two voices — your own unknowns, and what reads as review feedback from someone else (the date-of-birth note, the bottom-sheet challenge). Confirm which are which. If the second set came from clinical stakeholders, that's direct evidence for the visioning section and worth quoting.

The walkthrough videos are the portfolio asset. Nine silent screen recordings, 16 to 105 seconds each, at phone resolution. They need no editing to be usable — embed them directly, or pull stills. The VBC agenda, ambient scribe, and contextual alerts recordings are the three that carry the most on their own.

A thread worth pulling between the two case studies. In this vision the Briefing is four short bullets with inline links — genuinely brief. By the time it reached alpha it had become paragraphs of prose, which is the exact problem the second case study opens on. The scannable brief was in the original vision and got lost somewhere between vision and implementation. That is a strong, honest connective line: the first piece sets the intent, the second shows you fighting to recover it. Worth saying explicitly in one of them.

Second archive reviewed — thirteen more recordings. Highlights and where they belong:

Recording	Shows	Use in
Vision quick flow	Condensed end-to-end demo of the same vision	This piece — the short version to lead with
v3 design prototype	The four-section clinical agenda with per-item status labels and the evidence drawer	This piece — vision becoming product

Recording	Shows	Use in
ASLV swarm (two versions)	Grayscale content-first exploration; provenance line, med reconciliation, values with trend	**Case study 2**
Schedule / ASLV / scribe	The three surfaces working together	Either
Citations scaling pattern	Recorded working session, you presenting	Neither — see below
Autocomplete, loading and mixed data states, long-load	Edge cases, empty states, latency behavior	Optional depth
Post-MVP progressive disclosure	Grouping suspects and care gaps into single items	Case study 2, optional

Audio. Five have audio tracks and I can't transcribe them here — no speech-to-text available in this environment, and the models can't be fetched through the sandbox. If any contain narration you want reflected in the write-ups, a rough transcript or a summary from you would bring them in.

One to handle carefully. The citations recording is a working session with you on camera and at least one other participant visible. That's a different privacy question from a screen recording — get consent or crop before it goes anywhere public.

Dates needed. The record starts in March with the project plan already written, so the visioning happened before that and is currently undated. A rough month for the workshops and for the prototype would let the arc read chronologically.

Anonymization. Company and product unnamed; stakeholders by role. Metrics are the one exposure worth a second look — engagement and satisfaction figures are more identifying than anything else here. Consider directional phrasing ("engagement rose roughly ten points against a more ambitious target").

Relationship to the other case study. This is the lead piece. The Brief case study is the depth complement — its role section should point back here in one line rather than re-telling the arc. I'll trim that once this one is settled.

Visual assets.

Asset	Status
End-to-end vision strip	**Placed**
Ambient scribe progression	**Placed**
Clinical alert with trigger, guideline and evidence	**Placed**
Desktop EHR sidecar	**Placed**
From→To transformation table	Already in the prose
Workstream map sized by unknown and impact	Already in the prose
Core workflow at XS, MD, XL	**Export from Figma** — can't be saved from this environment
Desktop option explorations	**Export from Figma**
Fourteen prototypes converging to one	Build from the repo file record

Eleven more vision stills sit unplaced in the asset folder — agenda drawer, custom agenda item, brief with linked terms, draft note and coding, patient data, agentic entry point. Use them where a section needs more support.

Anonymization applies to the images too. The vision frames use an invented patient throughout, which is fine, but the desktop sidecar shows a full simulated record with named care-team members. Worth a pass before publishing.